

WEEK 1 RESEARCH • PAINMED-PA

# SLD

## Social Listening Dashboard

### From Public Signals to Actionable RCM Intelligence

WEEK 1 RESEARCH & DASHBOARD CONCEPTS

An external intelligence layer that detects meaningful payer, policy, procedure and RCM changes — before they become operational surprises.

**30**

Sources scored for acquisition feasibility

**1,946**

Records sampled across core-tested sources

**6**

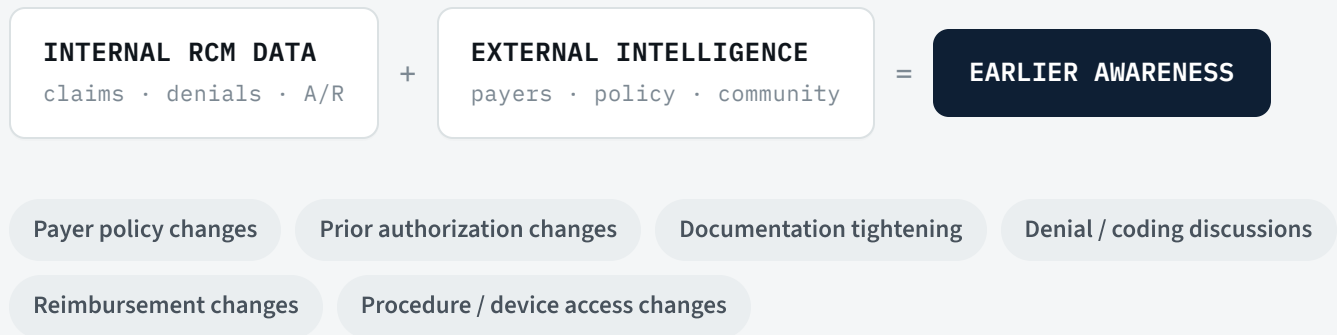
Master signal buckets

**3**

Dashboard concepts designed

# Practices can see inside. Not outside.

Internal RCM data — claims, denials, A/R — tells a practice what already happened. It doesn't say what's changing **outside** the practice: payer policy, prior authorization, documentation rules, reimbursement.



## Three roles. Three different questions.

USER

**CEO / Practice Owner**

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QUESTION

“What changed externally?”

DECISION

“Where should leadership focus?”

USER

**RCM / Billing Manager**

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QUESTION

“What changed operationally?”

DECISION

“What should I investigate?”

USER

**Account Team**

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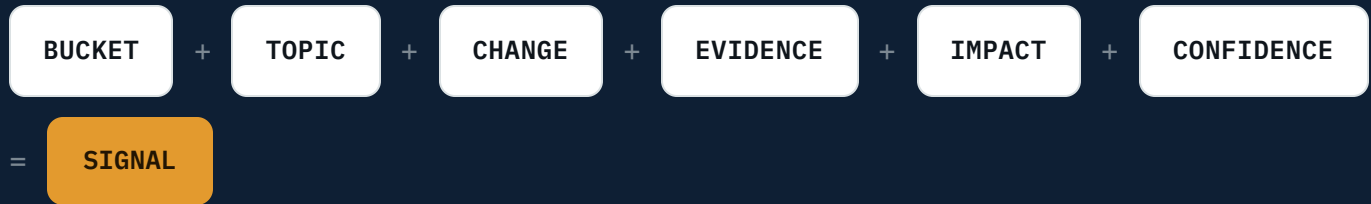
QUESTION

“Which clients may be affected?”

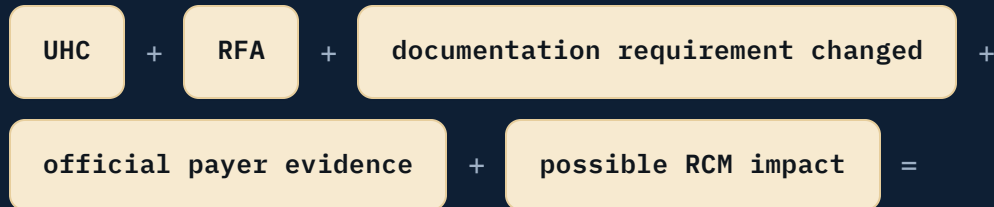
DECISION

“What should I discuss with them?”

# A mention is not a signal.



## WORKED EXAMPLE



**A mention is not automatically a signal.** A signal requires meaningful context and evidence — not just that a payer or procedure was named somewhere.

# Five checks before something becomes a signal

01

## Repetition

Are multiple people discussing the same issue?

02

## Velocity

Is discussion increasing?

03

## Independence

Are the reports from different people / sources?

04

## Specificity

Does it mention a payer / procedure / action?

05

## Novelty

Is this new compared with the baseline?

# Six master signal buckets

Every candidate signal is filed into one of six buckets. Tap a bucket to see its sub-types.

BUCKET 01

## Authorization / Utilization Management

10 sub-types · tap to expand

PA Requirement   PA Documentation  
PA Turnaround   PA Approval / Denial  
Additional Information   Peer-to-Peer  
Frequency Restriction   Site-of-Service  
Step Therapy   Utilization Restriction

BUCKET 02

## Denials / Claims Friction

9 sub-types · tap to expand

Denial Rate   New Denial Reason  
Documentation Denial  
Medical-Necessity Denial   Coverage Denial  
Coding Denial   No-Auth Denial  
Appeal Outcome   Claim Rework

BUCKET 03

## Coverage / Policy

8 sub-types · tap to expand

Coverage Change   Coverage Restriction  
New Exclusion   Policy Revision  
LCD / NCD Change   Effective-Date Change  
Provider Bulletin   Procedure / Device Coverage

BUCKET 04

## Documentation / Medical Necessity

8 sub-types · tap to expand

Documentation Requirement   Clinical Criteria  
Imaging Requirement   Conservative Treatment  
Clinical History   Physician Note  
Supporting Evidence  
Medical-Necessity Tightening

BUCKET 05

## Reimbursement / Payment

BUCKET 06

## Procedure / Device Access

8 sub-types · tap to expand

Rate Change

Fee Schedule

Contract Rate

Payment Rule

Underpayment

Payment Delay

Bundling

Modifier Payment

10 sub-types · tap to expand

Procedure Coverage

Procedure PA

Procedure Documentation

Procedure Medical Necessity

Procedure Denial

Frequency Restriction

Device Coverage

Device Authorization

Device Documentation

Device Denial

# We explored three different dashboard experiences for three different decisions.

## CONCEPT 01

### Executive Risk Radar

FOR · CEO / Practice Owner

“What has changed outside that could affect my practice?”

- One screen, one question: what should leadership pay attention to this week.
- Surfaces trending topics, top payers and top procedures by activity.
- Ranks recent signals by status and impact, not by raw mention count.

High Attention

Emerging Signals

Guideline Changes

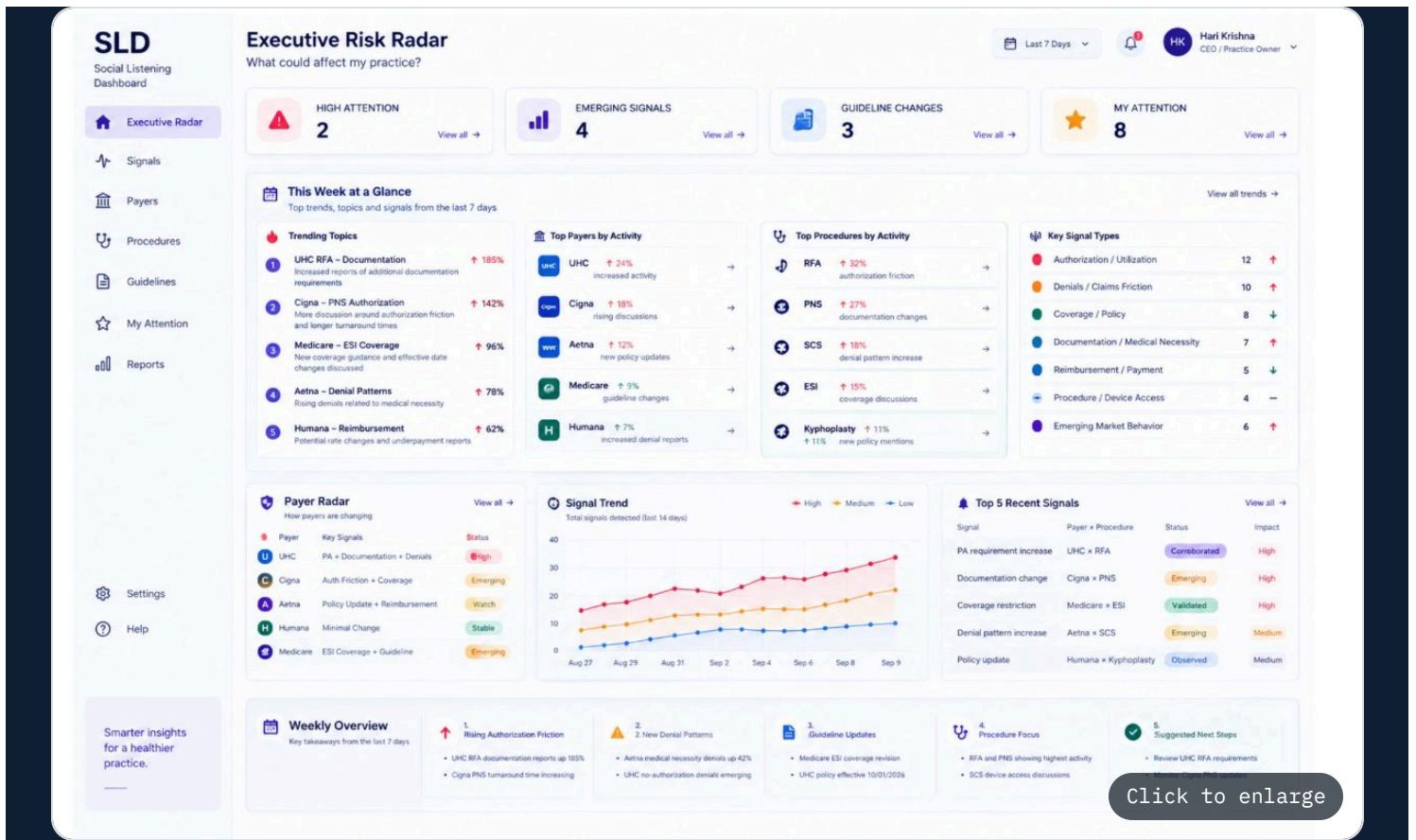
Payer Radar

Signal Trend

Recent Signals

Suggested Next Steps

**SIGNAL** · Payer + procedure + meaningful external change + evidence.



⌚ Numbers and trend lines shown in this mockup are concept/mock data for design review — not live measurements.

## CONCEPT 02

# RCM Change Center

FOR · RCM / Billing / Authorization Manager

“What changed and what should I investigate?”

- Built around a Payer × Procedure matrix — the working unit of RCM.
- Tracks changes since the last check, with a confidence score per signal.
- Turns each signal into a short, concrete "what should I check" checklist.

Changes Since Last Check

Active Signals

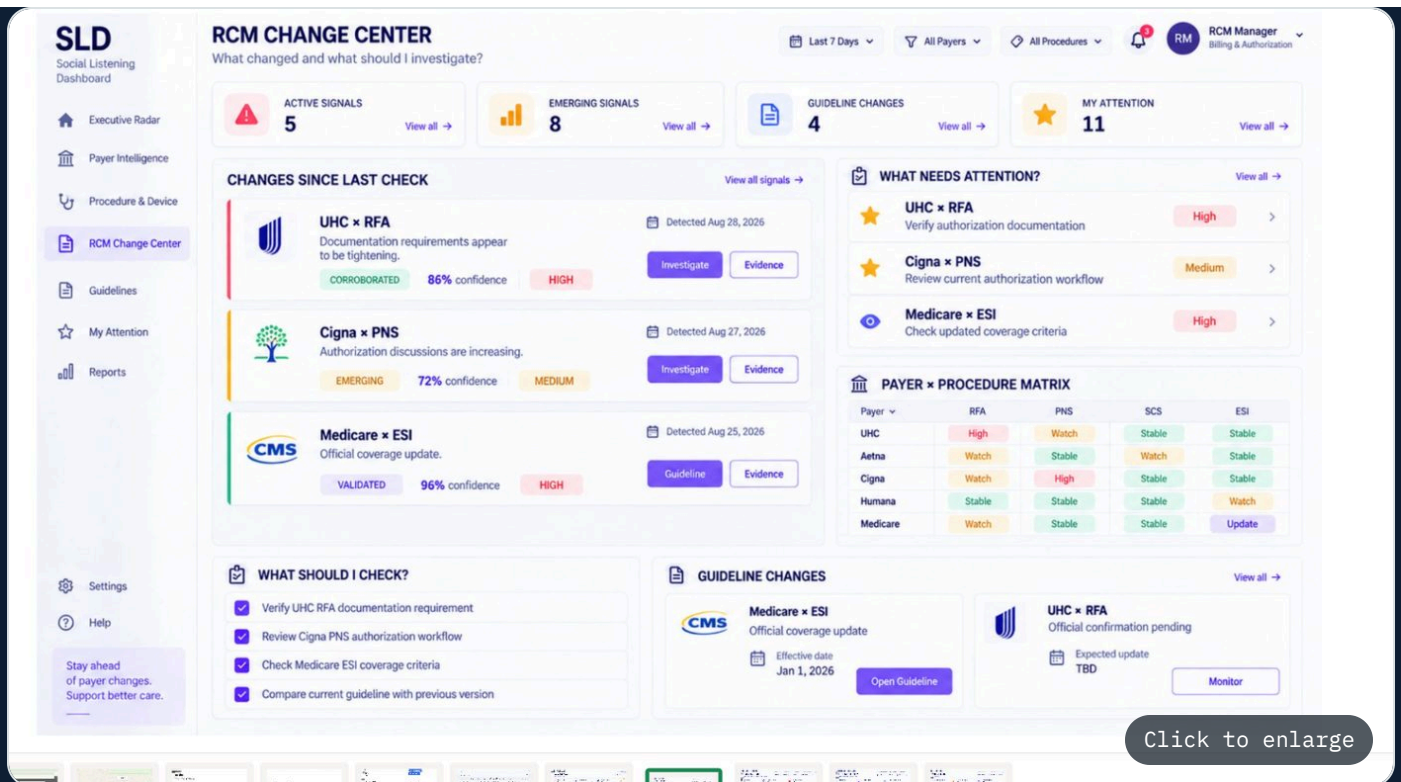
Emerging Signals

Payer × Procedure

What Needs Attention?

Guideline Changes

**SIGNAL** · A payer + procedure combination where public information shows a recent problem, policy change, or increasing discussion.



Numbers and trend lines shown in this mockup are concept/mock data for design review — not live measurements.

## CONCEPT 03

# Client Intelligence

FOR · PainMed-PA Account Team

“Which clients could be affected by what we’re seeing externally?”

- Reframes external signals around each client’s own payer / procedure mix.
- Gives account managers a ready-made talking point per client.
- Client x payer x procedure matching is the intended design goal — not yet an automated capability.

Clients Needing Attention

Emerging Client Risks

Followed Signals

Client x Payer x Procedure

Recent Signals

What Should I Discuss?

**SIGNAL** · Not yet verified as automated: intended to connect a signal to the specific clients it could affect.

# SLD

Social Listening  
Dashboard

- Executive Radar
- Payer Intelligence
- Procedure & Device
- Prior Authorization
- Denial Intelligence
- Client Intelligence
- Reports
- My Attention

Settings

Help

Helping practices  
stay ahead.  
Stronger together.

## CLIENT INTELLIGENCE

Which clients need attention?

Last 7 Days All Clients All Payers Account Team PainMed-PA

CLIENTS NEEDING ATTENTION

4

+1

vs previous 7 days

EMERGING CLIENT RISKS

7

+2

vs previous 7 days

GUIDELINE CHANGES

3

+1

vs previous 7 days

FOLLOWED SIGNALS

12

+3

vs previous 7 days

CLIENTS NEEDING ATTENTION

Clients with relevant external signals that may impact their practice.

View all clients →

Client A

Interventional Pain Group (FL)

UHC × RFA

HIGH

CORROBORATED 86% confidence

Why relevant?

Client performs RFA and works with UHC.

View Client

View Evidence

Client B

Spine & Pain Specialists (TX)

Cigna × PNS

MEDIUM

EMERGING 72% confidence

Why relevant?

Client performs PNS.

View Client

View Evidence

Client C

Advanced Pain Care (AZ)

Medicare × ESI

HIGH

VALIDATED 96% confidence

Why relevant?

Client has Medicare ESI exposure.

View Client

Guideline

CLIENT ATTENTION

Manage Attention

IMPORTANT

Client A

UHC × RFA

WATCH

Client B

Cigna × PNS

FOLLOW

Client C

Medicare

ALREADY DISCUSSED

Client D

UHC

| Client   | Practice Name                  | Payer    | Procedure | Current Status | Last Signal  | Action |
|----------|--------------------------------|----------|-----------|----------------|--------------|--------|
| Client A | Interventional Pain Group (FL) | UHC      | RFA       | High           | Aug 28, 2026 | View   |
| Client B | Spine & Pain Specialists (TX)  | Cigna    | PNS       | Medium         | Aug 27, 2026 | View   |
| Client C | Advanced Pain Care (AZ)        | Medicare | ESI       | Update         | Aug 25, 2026 | View   |
| Client D | Pain & Spine Center (GA)       | Aetna    | SCS       | Stable         | Aug 24, 2026 | View   |

RECENT SIGNALS

Latest relevant signals across your clients.

View all signals →

| Signal         | Topic         | Status       | Confidence | Clients Affected | Date         |
|----------------|---------------|--------------|------------|------------------|--------------|
| UHC × RFA      | Documentation | Corroborated | 86%        | Client A         | Aug 28, 2026 |
| Cigna × PNS    | Authorization | Emerging     | 72%        | Client B         | Aug 27, 2026 |
| Medicare × ESI | Coverage      | Validated    | 96%        | Client C         | Aug 25, 2026 |

WHAT SHOULD I DISCUSS?

Key talking points for your client conversations.

1 Ask Client A to verify UHC RFA requirements.

2 Mention Cigna PNS authorization trend to Client B.

3 Share Medicare ESI guideline update with Client C.

4 Monitor other clients for similar payer activity.

Click to enlarge

Numbers and trend lines shown in this mockup are concept/mock data for design review — not live measurements.

## Before sources: what do these dashboards actually need?

| DASHBOARD    | MAIN INFORMATION NEEDED                                          |
|--------------|------------------------------------------------------------------|
| CEO          | Payer changes, emerging issues, policy changes, procedure trends |
| RCM Manager  | Payer × Procedure, PA, denials, documentation, reimbursement     |
| Account Team | Client × Payer × Procedure, relevant external signals, evidence  |

Now that we know what each dashboard needs — the question becomes: **where can we actually get this information?**

# Different sources answer different questions

## CONFIRMATION

### Government & Official Medicare Sources

CMS MCD

CMS Coverage API

CMS Data APIs

Noridian

Novitas

Palmetto GBA

WPS

NGS

CGS

Federal Register

## POLICY GROUND TRUTH

### Commercial Insurance Payers

UHC

Aetna

Cigna

Humana

Elevance / Anthem

BCBS

## CORROBORATION

### Healthcare & RCM Professional Organizations

AAPC

MGMA

AMA

ASIPP

AAPM

HFMA

Becker's

Part B News

## EARLY DISCOVERY

### Online Communities & Social Platforms

r/CodingandBilling

r/MedicalCoding

r/MedicalBill

r/HealthInsurance

r/medicine

LinkedIn

X / Twitter

## AWARENESS / CONTEXT

### Healthcare Media & Educational Sources

YouTube

Student Doctor Network

### OFFICIAL SOURCES

→ confirmation

### PROFESSIONAL SOURCES

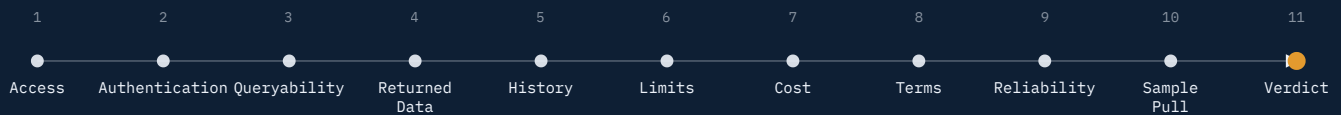
→ corroboration / context

### SOCIAL SOURCES

→ early discovery

# What we checked before trusting a source

Every source went through the same 11-point feasibility pass.



Every source moves left to right through this checklist before it earns a verdict.

## WORKED EXAMPLE • UHC

ACCESS Web / PDF monitoring

TERMS Restrictive policy content

RELIABILITY High

VERDICT ● BUILD NOW

## LEGEND

● BUILD NOW / USE

● SELECTIVE / VALIDATE

● DEFER

Full detail for every source — access, cost, terms, sample results — is in the next section.

# 30 sources, scored against that checklist

The two tables from the research doc, reproduced directly.

Table 1 · Source Summary

| SOURCE | REAL RELEVANT EXAMPLE FOUND                                                                                                                                                                       | WHAT IT GIVES SLD                                                                          | MEASURED SAMPLE                                                                                  |
|--------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------|
| AAPC   | Billing/Reimbursement forum contains current payer/denial discussions such as “BCBS Tx denying CPT codes 15771 & 15772,” “UHC Medicare denying 76881,” and “Horizon NJ Health Downcoding Claims.” | Real billing, denial, coding and payer experiences from the RCM/billing community          | 100 sampled → 3<br>Direct, 48<br>Supporting, 19<br>Irrelevant → 33<br>Direct / 81%<br>Broad      |
| MGMA   | Sept. 2026 MGMA Stat found 44% of medical-group leaders said payer PA turnaround became slower in 2026, while fewer than 1 in 10 saw faster turnaround.                                           | Practice-management, PA, RCM and payer-behavior trends                                     | 92 sampled → 23<br>Direct, 26<br>Supporting, 43<br>Irrelevant →<br>25.0% Direct /<br>53.3% Broad |
| AMA    | May 2026 survey of 1,000 physicians found only 33% believed insurer PA pledges would make a meaningful difference. AMA also tracks PA reform and insurer commitments.                             | Physician experience, PA trends, payer-policy context and corroboration                    | 107 sampled → 4<br>Direct, 102<br>Supporting, 1<br>Irrelevant →<br>3.7% Direct /<br>99.1% Broad  |
| ASIPP  | ASIPP reported on the final peripheral-nerve LCDs from five MACs, including CGS, Palmetto, Noridian, Wellpoint Federal and WPS, with policies taking effect Oct. 25, 2026.                        | Interventional-pain-specific coverage, LCD, Medicare, payer and reimbursement intelligence | 103 sampled → 4<br>Direct, 30<br>Supporting, 31<br>Irrelevant →<br>40.8% Direct /<br>69.9% Broad |
|        |                                                                                                                                                                                                   |                                                                                            |                                                                                                  |

| SOURCE                          | REAL RELEVANT EXAMPLE FOUND                                                                                                                                                                                         | WHAT IT GIVES SLD                                                         | MEASURED SAMPLE                                                                   |
|---------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| <b>AAPM</b>                     | Pain-medicine professional content includes policy, CMS and procedure-specific material.                                                                                                                            | Pain-medicine specialty context, policy and clinical/coverage information | 41 sampled → 17 Direct, 15 Supporting, 9 Irrelevant → 41.5% Direct / 78.0% Broad  |
| <b>HFMA</b>                     | HFMA/Guidehouse RCM research reported by Becker's found 88% of provider executives cited payer challenges among their top three RCM stressors, with 81% reporting increased denials and 74% increased PA delays.    | RCM trends, denial trends, payer friction and financial impact            | 98 sampled → 24 Direct, 47 Supporting, 27 Irrelevant → 24.5% Direct / 72.4% Broad |
| <b>Becker's Hospital Review</b> | 2026 reporting covers payer challenges, denials, PA delays, underpayments and reimbursement. One 2026 report found 88% of surveyed provider executives ranked payer challenges among their top three RCM stressors. | Industry news, payer trends, reimbursement and emerging RCM issues        | 138 sampled → 1 Direct, 93 Supporting, 27 Irrelevant → 13.0% Direct / 80.4% Broad |
| <b>Part B News</b>              | Specialized Medicare Part B content covers billing, coding, reimbursement and procedure-specific Medicare issues.                                                                                                   | Detailed Medicare Part B billing/coverage intelligence                    | 29 sampled → 14 Direct, 8 Supporting, 7 Irrelevant → 48.3% Direct / 75.9% Broad   |

Table 3 · Cost Analysis

| # | SOURCE                                  | TYPE              | RECOMMENDED ACQUISITION | A                       |
|---|-----------------------------------------|-------------------|-------------------------|-------------------------|
| 1 | <b>CMS — Medicare Coverage Database</b> | Official Medicare | API / download          | N<br>p                  |
| 2 | <b>CMS Coverage API</b>                 | Official API      | Official API            | N<br>li<br>e<br>n<br>to |

| #  | SOURCE                   | TYPE             | RECOMMENDED ACQUISITION | ACQUISITION                |
|----|--------------------------|------------------|-------------------------|----------------------------|
| 3  | CMS Data APIs            | Government data  | API                     | Un                         |
| 4  | Noridian                 | MAC              | Web + CMS API           | N<br>p<br>p                |
| 5  | Novitas                  | MAC              | Web + CMS API           | N<br>p<br>p                |
| 6  | Palmetto GBA             | MAC              | Web / PDF + CMS API     | N<br>p                     |
| 7  | WPS                      | MAC              | Public JSON + web       | N<br>p                     |
| 8  | NGS                      | MAC              | Web / PDF + CMS MCD     | P<br>p<br>a<br>v           |
| 9  | CGS                      | MAC              | Web + CMS MCD           | N<br>p<br>p                |
| 10 | UnitedHealthcare (UHC)   | Commercial payer | Web / PDF monitoring    | N<br>p<br>p                |
| 11 | Aetna                    | Commercial payer | Web extraction          | N<br>C                     |
| 12 | Cigna                    | Commercial payer | Web extraction          | P<br>p<br>l<br>c<br>f<br>r |
| 13 | Humana                   | Commercial payer | Web extraction          | N<br>s                     |
| 14 | Elevance Health / Anthem | Commercial payer | Web / PDF               | N<br>p                     |
|    |                          |                  |                         |                            |

| #  | SOURCE                        | TYPE                    | RECOMMENDED ACQUISITION           | APPROPRIATE FOR          |
|----|-------------------------------|-------------------------|-----------------------------------|--------------------------|
| 15 | Blue Cross Blue Shield (BCBS) | Commercial payer family | Plan-specific web extraction      | Unlabeled                |
| 16 | AAPC                          | Professional / forum    | Web extraction                    | Not for publication      |
| 17 | MGMA                          | Professional            | Web extraction                    | Source code repository   |
| 18 | AMA                           | Professional            | Web / PDF                         | Publication              |
| 19 | ASIPP                         | Pain specialty          | Web / PDF                         | Not for publication      |
| 20 | AAPM                          | Pain specialty          | Web / PDF                         | Not for publication      |
| 21 | HFMA                          | RCM professional        | Web / manual                      | Source code repository   |
| 22 | Becker's Hospital Review      | Trade media             | Web extraction                    | Source code repository   |
| 23 | Part B News                   | Medicare trade media    | Web / manual                      | Source code repository   |
| 24 | r/CodingandBilling            | Community               | Reddit API / approved third party | Approved for publication |
| 25 | r/MedicalCoding               | Community               | Reddit API / approved third party | Approved for publication |
| 26 | r/MedicalBill                 | Community               | Reddit API / approved third party | Approved for publication |
| 27 | r/HealthInsurance             | Community               | Reddit API / approved third party | Approved for publication |
|    |                               |                         |                                   |                          |

| #  | SOURCE             | TYPE                  | RECOMMENDED ACQUISITION            | A           |
|----|--------------------|-----------------------|------------------------------------|-------------|
| 28 | <b>r/medicine</b>  | Community             | <b>Apify (reddit-scraper-lite)</b> | A<br>o<br>p |
| 29 | <b>LinkedIn</b>    | Social / professional | <b>Apify scrapers</b>              | O<br>a<br>a |
| 30 | <b>X / Twitter</b> | Social                | <b>apidojo/tweet-scraper</b>       | A<br>c      |

## What we actually found

Six real examples pulled straight from testing, spanning the taxonomy.

|                |                                                                                                                                                                               |
|----------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| SOURCE         | AAPC · forum thread                                                                                                                                                           |
| OBSERVATION    | Provider asks why UHC rejected claims 64483/64484 for missing modifiers; replies cite CMS NCD article 56681 and UHC’s own bilateral-procedures PDF (RT / LT / 50 / KX rules). |
| SIGNAL TYPE    | Documentation Requirement · Coding Denial                                                                                                                                     |
| PAYER / TOPIC  | UHC · Nerve root block / epidural injection                                                                                                                                   |
| WHY IT MATTERS | Shows discovery (community) and validation (official CMS + payer policy) working end to end on the same thread.                                                               |

**DISCOVERY → VALIDATED**

|                |                                                                                                                                 |
|----------------|---------------------------------------------------------------------------------------------------------------------------------|
| SOURCE         | AAPC · forum                                                                                                                    |
| OBSERVATION    | Quoted threads: “UHC Medicare denying 76881”, “BCBS Tx denying CPT codes 15771 & 15772”, “Horizon NJ Health downcoding claims.” |
| SIGNAL TYPE    | New Denial Reason · Coding Denial                                                                                               |
| PAYER / TOPIC  | UHC · BCBS · Horizon NJ Health                                                                                                  |
| WHY IT MATTERS | Frontline billing staff surfacing payer-specific denial patterns in near real time.                                             |

● **SUPPORTING / CORROBORATING**

|             |                                                                                                                                        |
|-------------|----------------------------------------------------------------------------------------------------------------------------------------|
| SOURCE      | ASIPP                                                                                                                                  |
| OBSERVATION | Reported the final peripheral-nerve LCDs from five MACs — CGS, Palmetto, Noridian, Wellpoint Federal and WPS — effective Oct 25, 2026. |
| SIGNAL TYPE | LCD / NCD Change                                                                                                                       |

|               |                                                                                 |
|---------------|---------------------------------------------------------------------------------|
| SOURCE        | AAPM                                                                            |
| OBSERVATION   | “Seniors Act and Medicare Advantage Prior Authorization” — posted Sept 4, 2026. |
| SIGNAL TYPE   | PA Requirement                                                                  |
| PAYER / TOPIC | Medicare Advantage                                                              |

PAYER / TOPIC Medicare (multi-MAC) · Peripheral Nerve Stimulation

WHY IT MATTERS A pain specialty org flagging a binding coverage change ahead of its effective date — high evidence.

**AUTHORITATIVE / VALIDATION**

WHY IT MATTERS Pain-medicine-specific PA tracking, low volume but high specificity.

● **SUPPORTING (SPECIALTY, LOW VOLUME)**

SOURCE MGMA Stat

OBSERVATION Sept 1, 2026 poll, 178 respondents: 44% said PA turnaround got slower in 2026; 90% said PA burden increased over the previous 12 months; Medicare Advantage ranked most burdensome.

SIGNAL TYPE PA Turnaround

PAYER / TOPIC Medicare Advantage · commercial · Medicaid

WHY IT MATTERS Disclosed methodology corroborates an operational burden trend at scale.

● **SUPPORTING (PROFESSIONAL, SURVEY-BASED)**

SOURCE HFMA / Becker's

OBSERVATION Guidehouse RCM research: 88% of provider executives ranked payer challenges among their top 3 RCM stressors; 81% reported more denials; 74% reported more PA delays.

SIGNAL TYPE Denial Rate · PA Turnaround

PAYER / TOPIC Broad / not pain-specific

WHY IT MATTERS Strong directional corroboration of the two headline problems at industry level — use as macro context, the research found almost no pain-specific content here.

● **SUPPORTING (BROAD RCM CONTEXT)**

## Counts of Direct + Supporting items only

Payer & Government

Professional & RCM

Communities (Reddit)

| SOURCE           | TOTAL ACTIVITIES           | AUTH / UM                 | DENIALS                   | COVERAGE                   | DOCUMENTATION             | REIMBU                 |
|------------------|----------------------------|---------------------------|---------------------------|----------------------------|---------------------------|------------------------|
| UHC              | <div><div></div></div> 100 | <div><div></div></div> 5  | <div><div></div></div> 0  | <div><div></div></div> 75  | <div><div></div></div> 0  | <div><div></div></div> |
| Aetna            | <div><div></div></div> 100 | <div><div></div></div> 0  | <div><div></div></div> 0  | <div><div></div></div> 100 | <div><div></div></div> 0  |                        |
| Cigna            | <div><div></div></div> 100 | <div><div></div></div> 11 | <div><div></div></div> 0  | <div><div></div></div> 100 | <div><div></div></div> 7  |                        |
| Humana           | <div><div></div></div> 93  | <div><div></div></div> 0  | <div><div></div></div> 0  | <div><div></div></div> 93  | <div><div></div></div> 0  |                        |
| Noridian         | <div><div></div></div> 100 | <div><div></div></div> 0  | <div><div></div></div> 0  | <div><div></div></div> 100 | <div><div></div></div> 92 |                        |
| Novitas          | <div><div></div></div> 100 | <div><div></div></div> 0  | <div><div></div></div> 0  | <div><div></div></div> 100 | <div><div></div></div> 89 |                        |
| CMS              | <div><div></div></div> 100 | <div><div></div></div> 0  | <div><div></div></div> 0  | <div><div></div></div> 100 | <div><div></div></div> 88 |                        |
| Federal Register | <div><div></div></div> 100 | <div><div></div></div> 5  | <div><div></div></div> 3  | <div><div></div></div> 14  | <div><div></div></div> 3  | <div><div></div></div> |
| LinkedIn         | <div><div></div></div> 95  | <div><div></div></div> 58 | <div><div></div></div> 37 | <div><div></div></div> 7   | <div><div></div></div> 9  |                        |

**These categories are multi-label** — one record can belong to more than one bucket, so row totals can exceed the source's total activity count. A high Coverage / Policy count doesn't mean a payer has many problems — it reflects the type of records that source publishes.

## How the sources feed the dashboards

| SOURCE             | EXECUTIVE RISK RADAR | RCM CHANGE CENTER | CLIENT INTELLIGENCE | OFFICIAL |
|--------------------|----------------------|-------------------|---------------------|----------|
| CMS                | ● PRIMARY            | ● PRIMARY         | ● PRIMARY           | ● YES    |
| WPS                | ● PRIMARY            | ● PRIMARY         | ● PRIMARY           | ● YES    |
| Palmetto GBA       | ● PRIMARY            | ● PRIMARY         | ● PRIMARY           | ● YES    |
| AAPC               | ● PRIMARY            | ● PRIMARY         | ● PRIMARY           | ● NOT    |
| Reddit communities | ● PRIMARY            | ● PRIMARY         | ● PRIMARY           | ● NOT    |
| ASIPP              | ● SUPPORTING         | ● SUPPORTING      | ● SUPPORTING        | ● NOT    |
| AAPM               | ● SUPPORTING         | ● SUPPORTING      | ● SUPPORTING        | ● NOT    |
| MGMA               | ● SUPPORTING         | ● SUPPORTING      | ● SUPPORTING        | ● NOT    |
| AMA                | ● SUPPORTING         | ● SUPPORTING      | ● SUPPORTING        | ● NOT    |

● PRIMARY / DIRECT

● SUPPORTING / CORROBORATING

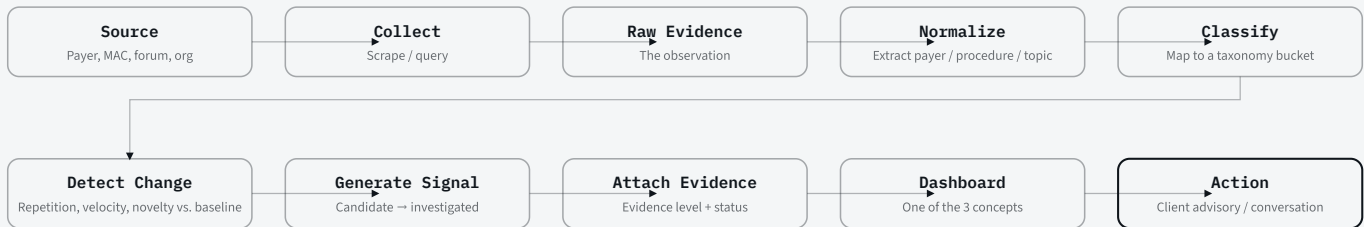
● NOT A DIRECT SOURCE

Only CMS, WPS and Palmetto GBA — the official / MAC sources — can supply **Official Confirmation** in signal investigation. Everything else corroborates.

PAYER & MAC SOURCES · UHC, AETNA, CIGNA, HUMANA, NORIDIAN, NOVITAS, ELEVANCE/ANTHEM, BCBS

All scored **● BUILD NOW** — strong for Coverage/Policy, Reimbursement and Procedure/Device signals. But per Section 12, their public content shows almost **zero Denials / Claims-Friction** activity — that gap is exactly what community and professional sources are for.

# How raw evidence becomes a signal



Raw evidence moves through ten stages before it reaches a dashboard as an actionable signal.

## EVIDENCE LEVELS

|                                           |             |
|-------------------------------------------|-------------|
| One patient Reddit post                   | Low         |
| Multiple patient reports                  | Low-Medium  |
| Professional billing discussion           | Medium      |
| Multiple independent professional reports | Medium-High |
| Specialty organization                    | High        |
| CMS / MAC / payer official policy         | Very High   |

## SIGNAL STATUS LADDER

|                     |                                                 |
|---------------------|-------------------------------------------------|
| <b>Observed</b>     | One relevant observation detected               |
| <b>Emerging</b>     | Pattern beginning to appear                     |
| <b>Corroborated</b> | Multiple independent sources support it         |
| <b>Validated</b>    | Authoritative source confirms the change        |
| <b>Active</b>       | Confirmed change currently affecting operations |
| <b>Resolved</b>     | No longer active / superseded                   |

## Latest → Daily → Weekly → Monthly

### LATEST

Newest signals detected since the last successful check.

### DAILY

What changed since the last successful check?

### WEEKLY

What is emerging across the sources we track?

### MONTHLY

What themes are recurring or becoming significant?

Historical data is primarily useful for establishing a **baseline** and detecting **change** against it — not as an end in itself.

## Six things the testing phase taught us

**01**

### **Official sources confirm.**

CMS, MACs and payer policy pages are strongest for confirmation — not discovery.

**02**

### **Professional orgs give context.**

AAPC, MGMA, AMA and specialty societies add operational and clinical framing.

**03**

### **Social sources discover early.**

Reddit surfaces problems first, but is noisier and needs corroboration.

**04**

### **Different sources, different signals.**

Payer sites returned ~0 denial content; that's the community's job, not theirs.

**05**

### **Access matters as much as relevance.**

LinkedIn and X were deferred on terms/access risk, not on topical fit.

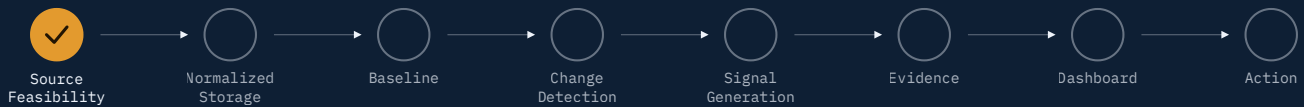
**06**

### **A mention isn't evidence.**

A useful signal needs repetition, velocity, independence, specificity and novelty — not just a name-drop.

## From Research → Continuous Intelligence

The next stage moves from source testing into periodic collection, change detection and signal generation. Nothing past feasibility is built yet.



Only “Source Feasibility” is complete. Everything after it is planned, not yet built.

## From Public Information to Actionable RCM Intelligence

“Know what changed.”

“Understand why it matters.”

“Know what to investigate next.”

